

Audit-grade RADV. 99% precision.

CMS is now auditing **every eligible Medicare Advantage contract, every year**, with multiple payment years in flight at once. Martlet AI verifies your submitted codes against the record, links the evidence to the page, and builds the submission packets — **for the audit you're in now, and every one after.**

99%

precision on
automated codes

95%

of codes closed
automatically

100k+

lives running
on the platform

Zero

PHI leaving
your network

What you actually get.

Every code checked against the record

Every code comes back **supported with evidence, flagged for deletion, or routed to your team as an exception** — with the page, the date and the signature behind the decision.

Your coders review the exceptions

95% of codes arrive already decided. The rest reach your team as exceptions, with the evidence attached and ranked. They don't have to go find the chart first.

Submission packets built as you go

Evidence sentence, source page, date of service, provider, signature, MEAT signals, coversheet.
Generated per HCC as you go — not assembled by hand inside a five-month window.

Runs on your own infrastructure

On-premise, in your private cloud, or air-gapped. **No outsourced reviewers, no chart shipping, no PHI egress** — and a full audit trail on every decision, replayable years later.

What gets checked on every chart.

The same set of checks runs on every chart — before submission, and again when the packet is assembled.

01

Document and encounter type

Every encounter inside a bundled file is identified and classified on its own.

02

Signature validity

Each record is checked for a valid signature, attributable to the clinician who wrote it.

03

Provider credentials

Signing clinicians are resolved against the current CMS specialty list. Gaps are flagged.

04

Dates of service

A date of service is established per encounter and checked against the period that applies.

05

Labs, imaging and pathology

Diagnostic reports are assessed under the criteria that apply to each type.

06

Clinical support

Each diagnosis is checked for support in the encounter itself, not carried forward.

CMS guidance changes. We update the checks to match, so validation follows the rules currently in force.

Four levels of review, in one workflow.

Findings arrive in the system your coders already work in. **Four levels by default, configured to your existing process — and every decision is recorded at each one.**



Level 01

Automated validation

Every check runs and each code is scored. **95% close here** — supported, or flagged.



Level 02

Certified coder

Exceptions only, with the evidence already attached and ranked.



Level 03

QA and audit lead

A sample re-reviewed for consistency, with **recurring patterns reported**.



Level 04

Compliance sign-off

Final approval, recorded against the evidence as it stood.

Once recorded, a decision cannot be altered or removed · every one shows who made it, on what evidence, and which version produced it.

In production at health systems, plans and platforms.



Before the letter. After the letter.

Once an audit begins, the remedies available to you are narrow. Which is why the work that matters happens before you are selected — and why the work after has to be fast.

Proactive — before the letter

Mock audits, at any scale.

- Sample your contracts on **CMS's own sampling methodology**, so your internal findings line up with how CMS will sample.
- Or verify every submitted HCC across the contract — **you can't predict which enrollees CMS draws**.
- Re-run quarterly and trend confirmation and deletion rates by coder, vendor and provider group.
- Fix documentation **before you are selected**.

Reactive — after the letter

The window is five months.

- Ingest the enrollee list and map **every audited HCC to its best evidence in hours**.
- Rank candidate records by validation strength, so you submit the strongest one.
- Surface signature and credential gaps **while the CMS attestation is still available**.
- Generate coversheet-ready packets and track intake rejections inside the window.

Beyond this year's audit.

CMS moved to auditing every eligible contract, every year. The work you do for this audit does not have to be repeated from scratch next year.

Readiness stops being a project

Your exposure is current and known at all times, rather than reconstructed from scratch each time a letter arrives.

The capability stays in-house

Workflow, evidence and audit trail live inside your organization, tuned to your contracts, your coders and your thresholds. The setup carries from one payment year to the next.

Documentation improves every cycle

Failure patterns surface by provider group, vendor and condition. Fix them at the source and next year's sample can be cleaner than this year's — the gains compound.

No annual scramble for capacity

You stop buying emergency coder and vendor capacity every cycle, and stop paying rush rates for it.

How the engagement works.

An annual license

You pay a license fee for the year. **Not per chart, and not per code we validate.**

Priced to your size

The fee is based on how many contracts and lives you run, and **stays fixed for the year**. Add contracts or payment years without renegotiating.

Configured to your process

We set the review levels, thresholds and exports to match how your team works, and **connect to the systems they already use**.

Bring one contract. We'll run a mock RADV on it.

Sampled on CMS's methodology, validated at 99% precision, findings returned by HCC with evidence packets and an exposure estimate — inside your environment, on codes submitted years ago, by anyone, on any platform.

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